



COLORADO
Department of Health Care
Policy & Financing



Colorado Medical Assistance Program

**Health Care Claim Professional (837)
Transaction Standard Companion Guide**

**Companion to Health Care Claim
ASC X12N 837 005010X222
Implementation Guide**

June 2025

Disclosure Statement

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Preface

This companion guide to the Health Care Claims (837s) adopted under Health Insurance Portability and Accountability Act (HIPAA) clarifies and specifies the data content when exchanging electronically with the Department of Health Care Policy & Financing (the Department). Transmissions based on this companion guide, used in tandem with the **ASC X12N 837 005010X222 and the associated addendum 005010X222A1 implementation guides**, are compliant with both ASC X12 syntax and those guides. This companion guide is intended to convey information that is within the framework of the ASC X12N implementation guides adopted for use under HIPAA. The companion guide is not intended to convey information that in any way exceeds the requirements or usages of data expressed in the implementation guides.

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1. INTRODUCTION

The Health Insurance Portability and Accountability Act (HIPAA) of 1996 carries provisions for administrative simplification. This requires the Secretary of the Department of Health and Human Services (HHS) to adopt standards to support the electronic exchange of administrative and financial health care transactions primarily between health care providers and plans. HIPAA directs the Secretary to adopt standards for transactions to enable health information to be exchanged electronically and to adopt specifications for implementing each standard HIPAA serves to:

- Create better access to health insurance
- Limit fraud and abuse
- Reduce administrative costs

The HIPAA regulations at [45 CFR 162.915](#) require that covered entities not enter into transition partner agreement that would do any of the following:

- Change the definition, data condition, or use of a data element or segment in a standard
- Add any data elements or segments to the maximum defined data set
- Use any code or data elements that are marked “not used” in the standard’s implementation specification or are not in the standard’s implementation specifications
- Change the meaning or intent of the standards implementation specifications

SCOPE

The companion guide is to be used with, and to supplement the requirements in the HIPAA ASC X12 implementation guides, without contradicting those requirements. Implementation guides define the national data standards, electronic format, and values for each data element within an electronic transaction. The purpose of the companion guide is to provide trading partners with a guide to communicate information specific to the Colorado Medical Assistance Program that is required to successfully exchange transactions.

The companion guide is intended for the business and technical users, within or on behalf of trading partners, responsible for the testing and setup of electronic claim status request and response transactions to the fiscal agent on behalf of the Department.

OVERVIEW

This section of the companion guide will provide guidance for establishing a relationship with the Department for the business purpose of submitting Health Care Claims (837s).

ADDITIONAL INFORMATION

It is assumed that the trading partner has purchased and is familiar with the ASC X12 Type 3 Technical Report (TR3) being referenced in this companion guide.

2. GETTING STARTED

TRADING PARTNER REGISTRATION

Any entity intending to exchange electronic transactions with the Department must agree to the Trading Partner Agreement at the end of the trading partner profile process. A trading partner profile can be completed using the [Provider Web Portal](#).

Note: Providers must be enrolled and approved before registering as a trading partner.

The Web Portal and the Secure File Transfer Protocol (SFTP) will include the ability for file and report retrieval. Billing agents and clearinghouses will have the option of retrieving the transaction responses and reports themselves and/or allowing each individual provider the option of retrieval. The

trading partner will access the system using the assigned login and password. **Refer to the File Delivery and Retrieval System Vendor Interface Specifications on the [Electronic Data Interchange \(EDI\) Support](#) web page for more information.**

3. TESTING WITH THE PAYER

This section contains a detailed description of the testing phase.

Testing is required for Health Care Claims (837s).

Before exchanging production transactions with the Department, each trading partner must complete production authorization testing.

Trading partner testing includes HIPAA compliance testing as well as validating the use of conditional, optional, and mutually defined components of the transaction.

Trading partners are encouraged to submit 10 successful and unique submissions and receive the associated 999 (accepted) acknowledgement in response in order to obtain approval to promote to production.

Trading partner authorization testing is detailed in the Trading Partner Testing Packet for ASC X12 transactions available on the Colorado [Electronic Data Interchange \(EDI\) Support page](#).

Questions may be directed to the [Provider Services Call Center](#).

4. CONTACT INFORMATION

Visit the [Colorado Department of Health Care Policy & Financing's website](#) for general information.

ELECTRONIC DATA INTERCHANGE (EDI) SERVICES

Contact the [Provider Services Call Center](#) with any questions.

5. CONTROL SEGMENTS/ENVELOPES

ISA-IEA

This section describes the use of the interchange control segments. It includes a description of expected sender and receiver codes, authorization information, and delimiters. (See Section 9 Transaction-Specific Information below.)

GS-GE

This section describes the use of the functional group control segments. It includes a description of expected application sender and receiver codes. Also included in this section is a description concerning how the Department expects functional groups to be sent and how the Department will send functional groups. These discussions will describe how similar transaction sets will be packaged and the use of functional group control numbers. (See Section 8 Transaction-Specific Information below.)

ST-SE

This section describes the use of transaction set control numbers. (See Section 8 Transaction-Specific Information below.)

Transactions (ST-SE envelopes) are limited to a maximum of 5000 CLM segments.

6. ACKNOWLEDGEMENTS AND REPORTS

The acknowledgement process will create the TA1 and the 999 acknowledgements for the inbound transactions, as well as an HTML report.

- 999s will be returned in all cases.
- HTML reports will be returned only in case of errors.
- TA1s will be returned based on the ISA14 indicator.
 - If ISA14 = 1
 - Positive scenario: TA1 will be sent
 - Negative scenario: TA1 will be sent
 - If ISA14 = 0
 - Positive scenario: TA1 will not be sent
 - Negative scenario: TA1 will be sent

7. TRADING PARTNER AGREEMENTS

An Electronic Data Interchange (EDI) trading partner is defined as any customer of the Department (provider, billing service, software vendor, employer group, financial institution, etc.) that transmits to or receives electronic data from the fiscal agent on behalf of the Department.

Payers have EDI Trading Partner Agreements (TPA) that accompany the standard implementation guide to ensure the integrity of the electronic transaction process. The Trading Partner Agreement is related to the electronic exchange of information, whether the agreement is an entity or a part of a larger agreement, between each party to the agreement.

8. TRANSACTION-SPECIFIC INFORMATION

This section describes how ASC X12N implementation guides (IGs) adopted under HIPAA will be detailed with the use of a table. The tables contain a row for each segment that contains additional information not found in the IGs. That information can:

1. Limit the repeat of loops, or segments
2. Limit the length of a simple data element
3. Specify a sub-set of the IGs internal code listings
4. Clarify the use of loops, segments, composite and simple data elements
5. Any other information tied directly to a loop, segment, composite or simple data element pertinent to trading electronically with the Department

In addition to the row for each segment, one (1) or more additional rows are used to describe the usage for composite and simple data elements and for any other information. Notes and comments should be placed at the deepest level of detail. For example, a note about a code value should be placed on a row specifically for that code value, not in a general note about the segment.

All clients of the Department are considered “subscribers,” and no patient loops are allowed. The patient will always be the subscriber. See the implementation guide for additional information.

The Trading Partner ID (TPID) is the number that is assigned to the provider/submitter to uniquely identify their electronic transaction. This may also be referred to as the Electronic Claim Submission (ECS) number or TPID.

Health Care Claim Professional (837P)

Loop ID	Reference	Name	Codes	Notes/Comments
HEADER	ISA	Interchange Control Header		The ISA is a fixed-length record with fixed-length elements. All inbound files are constrained to a single ISA segment for tracking and balancing. Note: Deviating from the standard ISA element sizes will cause the Interchange to be rejected.
	ISA01	Authorization Information Qualifier	00	
	ISA02	Authorization Information		No data is expected in this data element.
	ISA03	Security Information Qualifier	00	
	ISA04	Security Information		No data is expected in this data element.
	ISA05	Interchange ID Qualifier	ZZ	
	ISA06	Interchange Sender ID		Enter the Trading Partner ID (TPID) assigned by the Colorado Medical Assistance Program.
	ISA07	Interchange ID Qualifier	ZZ	
	ISA08	Interchange Receiver ID	COMEDASSIST PROG	
	GS	Functional Group Header		
	GS02	Application Sender's Code		Enter the Trading Partner ID (TPID) assigned by the Colorado Medical Assistance Program.
	GS03	Application Receiver's Code	COMEDASSIST PROG	
	GS08	Version/Release/ Industry Identifier Code	005010X222A1	Standards Approved for Publication by ASC X12 Procedures Review Board.
	ST	Transaction Set Header		

Loop ID	Reference	Name	Codes	Notes/Comments
	ST03	Version, Release, or Industry Identifier	005010X222A1	
	BHT	Beginning of Hierarchical Transaction		
	BHT06	Claim or Encounter Identifier	CH, RP	For submitting fee-for-service claims, enter the following value: CH For submitting encounter claims, enter the following value: RP Note: Fee-for Service and encounter claims must be submitted in separate batches.
1000B	NM1	Receiver Name		
	NM103	Receiver Name	COLORADO MEDICAL ASSISTANCE PROGRAM	
	NM109	Receiver Primary Identifier	COMEDASSIST PROG	
2000B	SBR	Subscriber Information		
	SBR09	Claim Filing Indicator Code		For Medicare Crossover Claim, enter one of the following values: 16, MA, or MB Otherwise enter the following value: MC
2010BA	NM1	Subscriber Name		
	NM108	Identification Code Qualifier	MI	
	NM109	Subscriber Primary Identifier		Enter the Colorado Medical Assistance Program Client ID
2010BB	NM1	Payer Name		
	NM108	Identification Code Qualifier	PI	
	NM109	Payer Identifier	CO_TXIX	
2300	CLM	Claim Information		

Loop ID	Reference	Name	Codes	Notes/Comments
	CLM05-3	Claim Frequency Code	1, 7, 8	Only codes 1, 7, and 8 are recognized for Colorado Medical Assistance Program Claims processing. If code 7 or 8 is used, the Payer Claim Control Number (2300/REF02) must also be included.
	PWK	Claim Supplemental Information		Colorado Medical Assistance Program does not support the use of this segment at this time.
	REF	Referral Number		
	REF02	Referral Number		Needed for timely filing requirements, enter the previous Internal Control Number (ICN).
	REF	Payer Claim Control Number		
	REF02	Payer Claim Control Number		Enter the Colorado Medical Assistance Program Internal Control Number (ICN) for the claim that is being adjusted.
	REF	Claim Identifier For Transmission Intermediaries		Colorado Medical Assistance Program requires this segment to be used for all Encounter Claim submissions.
	REF02	Value Added Network Trace Number		The system does not store any alphabetic characters. If a TCN contains alphabetic characters, only the leading numeric values will be stored in interChange for query and reporting. If a TCN contains more than 17 numeric characters, the system will truncate the number to only retain the first 17 characters. - Ex: TCN received starts with a number – 123A45, the system will store and report 123 as the TCN for the encounter. - Ex: TCN received starts with alphabetic character =A12345, the system will not

Loop ID	Reference	Name	Codes	Notes/Comments
				store and report a TCN for the encounter. Ex: TCN received ="2227211813004333 867", the system will store "22272118130043338" Enter the Managed Care Entity Claim Number.
	REF	Medical Record Number		
	REF02	Medical Record Number		Maximum length of 30 bytes supported
2320	SBR	Other Subscriber Information		For MCO Encounter claims, the MCO will be considered an Other Payer and the MCO payment information should be included in the last iteration of this loop.
	SBR09	Claim Filing Indicator Code		Enter one of the following values, which correctly identifies the type of Medicare coverage provided by the policy: 16, MA, or MB For Managed Care Organization Encounter claims, enter "ZZ" to identify the submitting MCO as a Payer on the claim.
2320	CAS	Claim Level Adjustment		
	CAS01	Claim Adjustment Group Code	CO, CR, OA, PI, PR	Use PI only for encounters denied by the Health Plan. These encounters will receive a denied disposition in interChange to align with the Health Plan's determination.
2400	SV1	Professional Service		
	SV101-2	Procedure Code		If a drug is being billed, an appropriate J-code or other drug related HCPCS code must be used.
2430	CAS	Line Adjustment		

Loop ID	Reference	Name	Codes	Notes/Comments
	CAS01	Claim Adjustment Group Code	CO, CR, OA, PI, PR	Use PI only for encounters denied by the Health Plan. These encounter services will receive a denied disposition in interChange to align with the Health Plan's determination.

APPENDIX 1: Change Summary

Date	Change	Responsible Party
March 2017	Original Document	HPE
3/31/2017	Added New EDI Service Telephone Number	HPE
8/1/2017	Rebranding to DXC Technology	DXC Technology, formerly HPE
9/18/2017	Updated the notes/comments for REF02 (Value Added Network Trace Number), links in the various sections, and verbiage in the Certification and Testing Overview section.	DXC
2/23/2018	Updated the notes/comments for Header/ISA.	DXC
8/13/2021	Updating hyperlinks, update REF TCN methodology,	Gainwell Technologies, formerly DXC
10/26/2022	Added CO and Gainwell branding, updated hyperlinks and general cleanup	Gainwell Technologies
1/18/2023	Updated the notes/comments for REF02 (Value Added Network Trace Number)	Gainwell Technologies
3/2/2023	Added BHA payer information; updated Provider Web Portal links	Gainwell Technologies
12/18/2023	Removed BHA payer information. BHA information will be in a BHA-specific companion guide.	Gainwell Technologies
5/1/2025	Removed deprecated information	Gainwell Technologies
6/24/2025	Added information for CAS adjustments	Gainwell Technologies